DISCHARGE SUMMARY

Patient Name: Mary Johnson

Date of Birth: 1965-02-14

Admission Date: 2026-03-22

Discharge Date: 2026-03-29

Attending Physician: Dr. Robert Lee, MD — Endocrinology

PRIMARY DIAGNOSIS

Type 2 Diabetes Mellitus with uncontrolled hyperglycemia

SECONDARY DIAGNOSES

- Hypertension
- Dyslipidemia
- Dehydration

PROCEDURES PERFORMED

- Fasting blood glucose tracking: Arrival glucose 280 mg/dL
- Initiation of short-term insulin therapy for glucose stabilization
- Diabetes education modules: Glucose monitoring and carbohydrate counting
- Blood pressure monitoring and titration of antihypertensive medications
- Podiatry consult for diabetic foot check (No ulcers noted)
- Nutritional consultation for Type 2 Diabetes management

MEDICATIONS AT DISCHARGE

- Metformin 1000 mg — take 1 tablet by mouth twice daily — ongoing
- Glimepiride 2 mg — take 1 tablet by mouth once daily with breakfast — ongoing
- Insulin glargine 10 units — inject under skin once daily at bedtime — ongoing
- Lisinopril 20 mg — take 1 tablet by mouth once daily — ongoing (for hypertension)
- Atorvastatin 40 mg — take 1 tablet by mouth once daily — ongoing (for dyslipidemia)

ACTIVITY RESTRICTIONS

- Moderate physical activity as tolerated (e.g., walking 20-30 mins daily)
- Inspect feet daily for any redness, blisters, or cuts; use a mirror if needed
- Always wear properly fitting shoes; do not walk barefoot even indoors
- Check blood sugar before driving or operating heavy machinery

DIETARY RESTRICTIONS

- Low-carbohydrate, high-fiber diet (ADA diet)
- Avoid sugary beverages and high-sugar processed foods
- Limit alcohol intake; never drink on an empty stomach due to hypoglycemia risk
- Stay well hydrated: drink 6-8 cups of water per day

FOLLOW-UP APPOINTMENTS

- Endocrinology: Dr. Robert Lee — 2026-04-12 — A1c and medication review
- Diabetes Education: 2026-04-05 — follow-up nutrition and glucose log check

RED FLAG SYMPTOMS

Call 911 or go to the ER immediately if you experience:

- Blood sugar level above 400 mg/dL or "HI" reading on meter
- Severe low blood sugar (<70 mg/dL) with confusion, sweating, or tremors
- Non-healing wounds, sores, or significant redness on the feet or legs
- Blurred vision that comes on suddenly
- Extreme thirst and frequent urination despite taking medication
- Numbness or tingling in hands or feet
- Shortness of breath or chest pain

DISCHARGE CONDITION

Stable — hyperglycemia improved with medication adjustments. Patient education completed.